



lichenoid dermatosis

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LICHENOID DERMATOSES

are heterogeneous group of conditions that....

- Clinically resemble idiopathic LP (i.e. pruritic violaceous flat-topped papules).
- Histologically demonstrate a lichenoid tissue reaction = vacuolar alteration (hydropic degeneration) of epidermal basal cells + a band-like inflammatory infiltrate (mainly lymphocytes) in the papillary dermis.

Interface dermatitis:

vacular degeneration of dermoepidermal junction with inflammatory infiltrate

Lichenoid interface
dermatitis : (band like
inflammatory infiltrate

LP

L. STREPTUS

L. NITIDUS

LICHENOID drug eruption

Vacular interface
dermatitis: (perivascular
inflammatory infiltrate)

CVHD

PLEVA- PLC

EM

CT disease (SLE, DM)

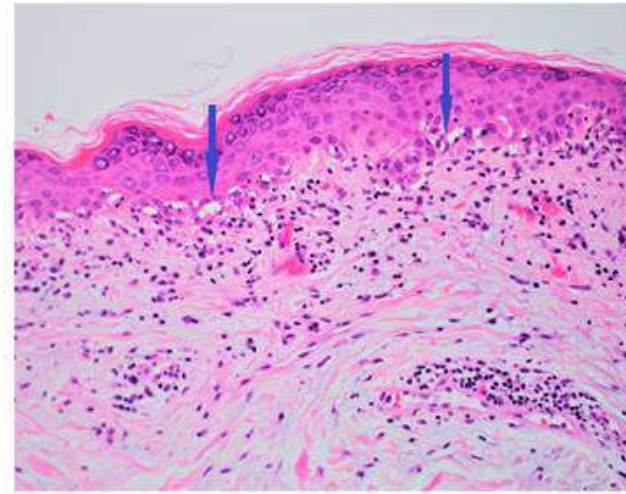
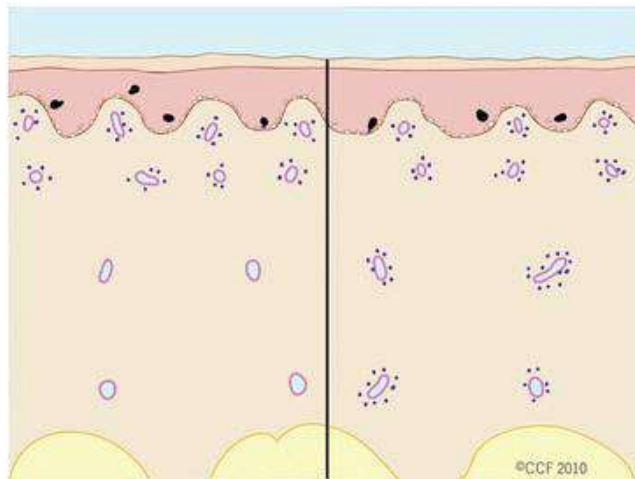
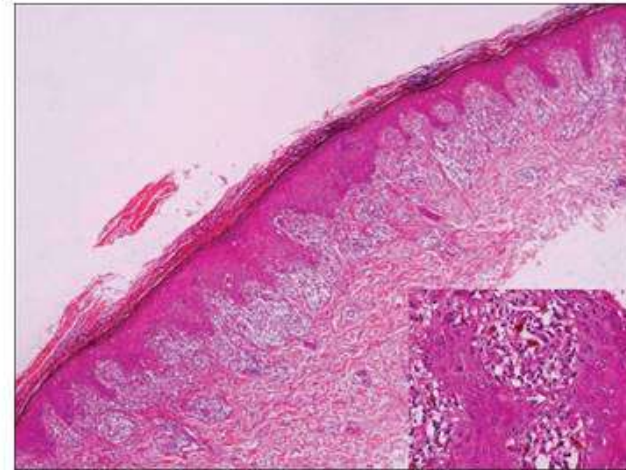
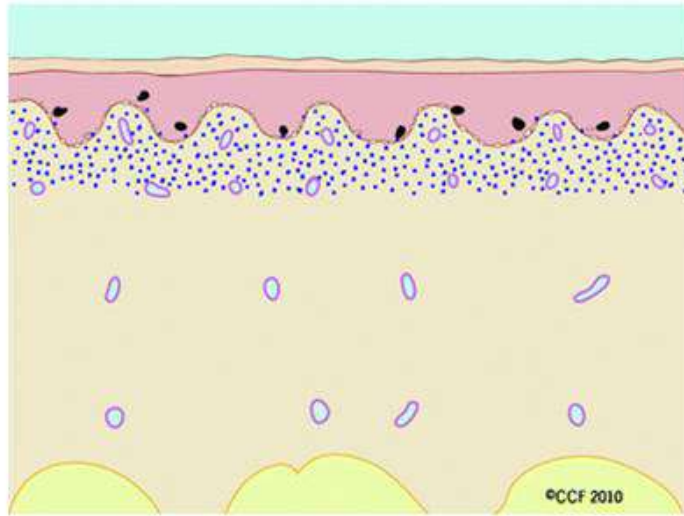
FDE

EM

MF

2R SYPHILIS

PARANEOPLASTIC PEMPHIGUS



LICHEN PLANUS :



LICHEN PLANUS

- Is idiopathic inflammatory disease of skin , mucous membrane , hair , nail .
- characterized
 - - o clinically by pruritic, violaceous papules that favor the extremities,
 - - o histologically by a dense, band-like lymphocytic infiltrate and destruction of the basal cell layer (vacuolar alteration)

EPIDEMIOLOGY

worldwide prevalence:

- Cutaneous LP: 0.2% -1 % of the adult population.
- Oral lesions: 1-4% of the population.
- Age of onset; fifth or sixth decade

AETIOPATHOGENESIS

Enviromental
triggers

In
Genaticly
susptable pt

Immune
dysregulation

Genetically-predisposed individuals + exposure to triggers —>



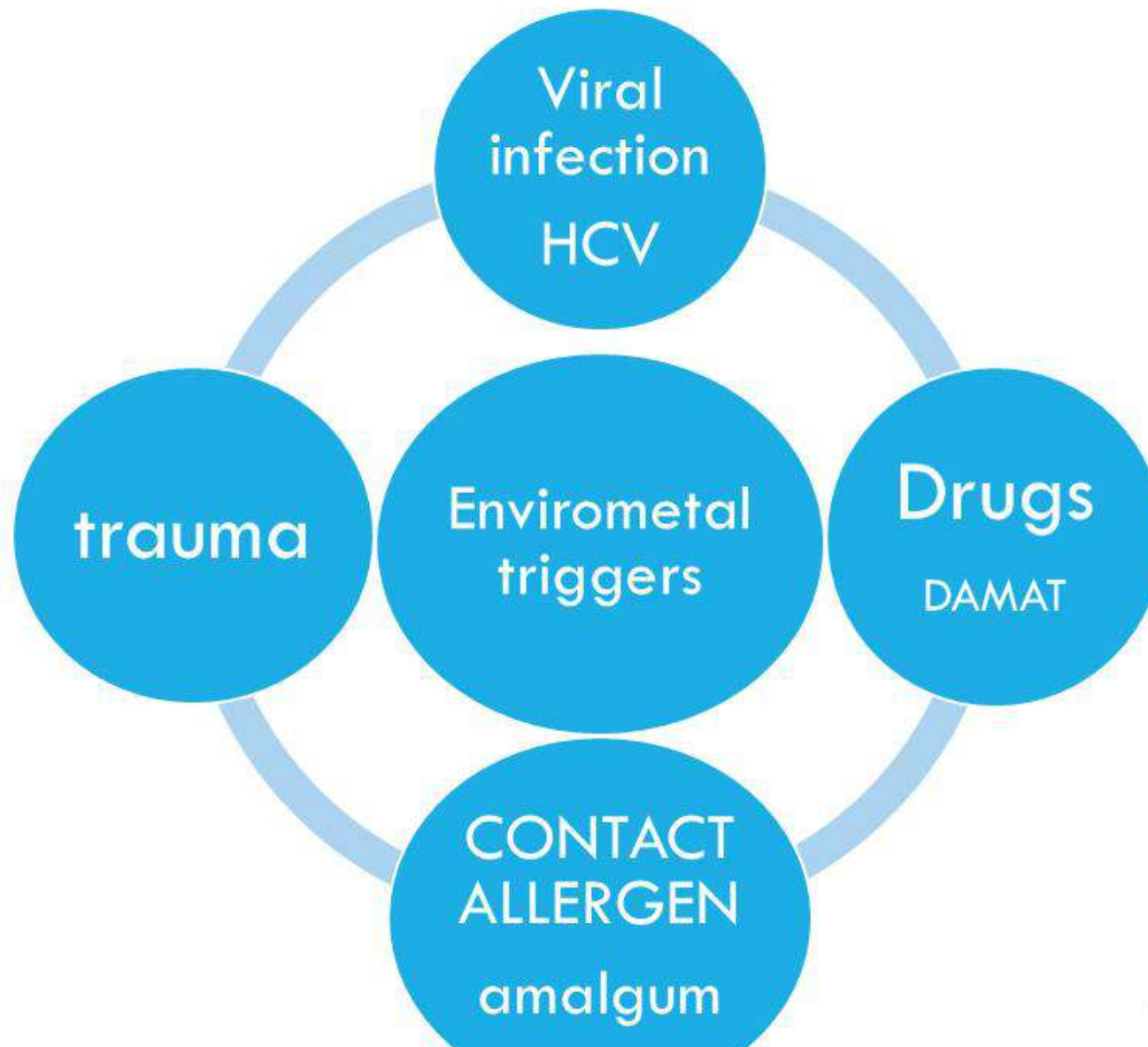
modification of keratinocytes —>



induction of immune response against them —▶



keratinocyte damage (CD8+ cytotoxic T-cells).



ENVIROMENTAL TRIGGERS

1. Viral infection : hcv , hsv , vzv , hhv6-7
2. Trauma : Koebner phenomena
3. Drug :
 1. DIURTICS
 2. ANTIHYPERTENSIVE : ACEI , B blocker
 3. Metals
 4. Antimalarials
 5. TNFinhibitors
4. Contact allergens :amalgam
5. Vaccines: HBV vaccine.
 - Bacteria: Helicobacter pylori.
 - Neoplasms:

DRUGS IMPLICATED IN LICHENOID DRUG ERUPTIONS	
Antimicrobials	
• Ethambutol	• Pyrimethamine
• Griseofulvin	• Streptomycin
• Isoniazid	• Sulfamethoxazole
• Ketoconazole	• Tetracyclines
Antihypertensives	
• Captopril	• Diazoxide*
• Enalapril	• Doxazosin
• Labetalol	• Nifedipine
• Methyldopa	• Prazosin
• Propranolol	
Antimalarials	
• Chloroquine	• Quinacrine
• Hydroxychloroquine	
Antidepressants, anti-anxiety drugs, antipsychotics and anticonvulsants	
• Amitriptyline	• Levomepromazine
• Carbamazepine	• Lorazepam
• Chlorpromazine	• Methopromazine
• Imipramine	• Phenytoin
TNF-α inhibitors	
• Etanercept	• Adalimumab
• Infliximab	• Lenercept
Diuretics	
• Chlorothiazide	• Furosemide
• Hydrochlorothiazide	• Spironolactone
Hypoglycemic agents	
• Chlorpropamide	• Tolazamide
• Glyburide	• Tolbutamide

Metals	
• Gold salts [‡]	• Mercury
• Arsenic	• Palladium
• Bismuth	
NSAIDs	
• Acetylsalicylic acid	• Ibuprofen
• Benoxaprofen	• Indomethacin
• Diflunisal	• Naproxen
• Fenclofenac	• Sulindac
• Flurbiprofen	
Miscellaneous drugs	
• Allopurinol	• Methycran
• Amiphenazole	• Nivolumab
• Anakinra	• Omeprazole
• Cinnarizine	• Orlistat
• Cyanamide	• Pembrolizumab
• Dapsone	• Penicillamine
• Gemfibrozil	• Procainamide
• Hydroxyurea	• Propylthiouracil
• Imatinib	• Pyridoxin
• Interferon- α	• Simvastatin
• Iodides	• Quinidine
• Isotretinoin	• Quinine
• Levamisole	• Rituximab
• Lithium	• Sildenafil
• Mercapto-propionylglycine	• Sulfasalazine
• Mesalamine	• Trihexyphenidyl
*Also used to treat hypoglycemia.	
[‡] Including in alcoholic beverages, e.g. Goldschläger.	

Table 11.2 Drugs implicated in lichenoid drug eruptions **KONNECT ACADEMY-DERMA - BY DR. EMAN**

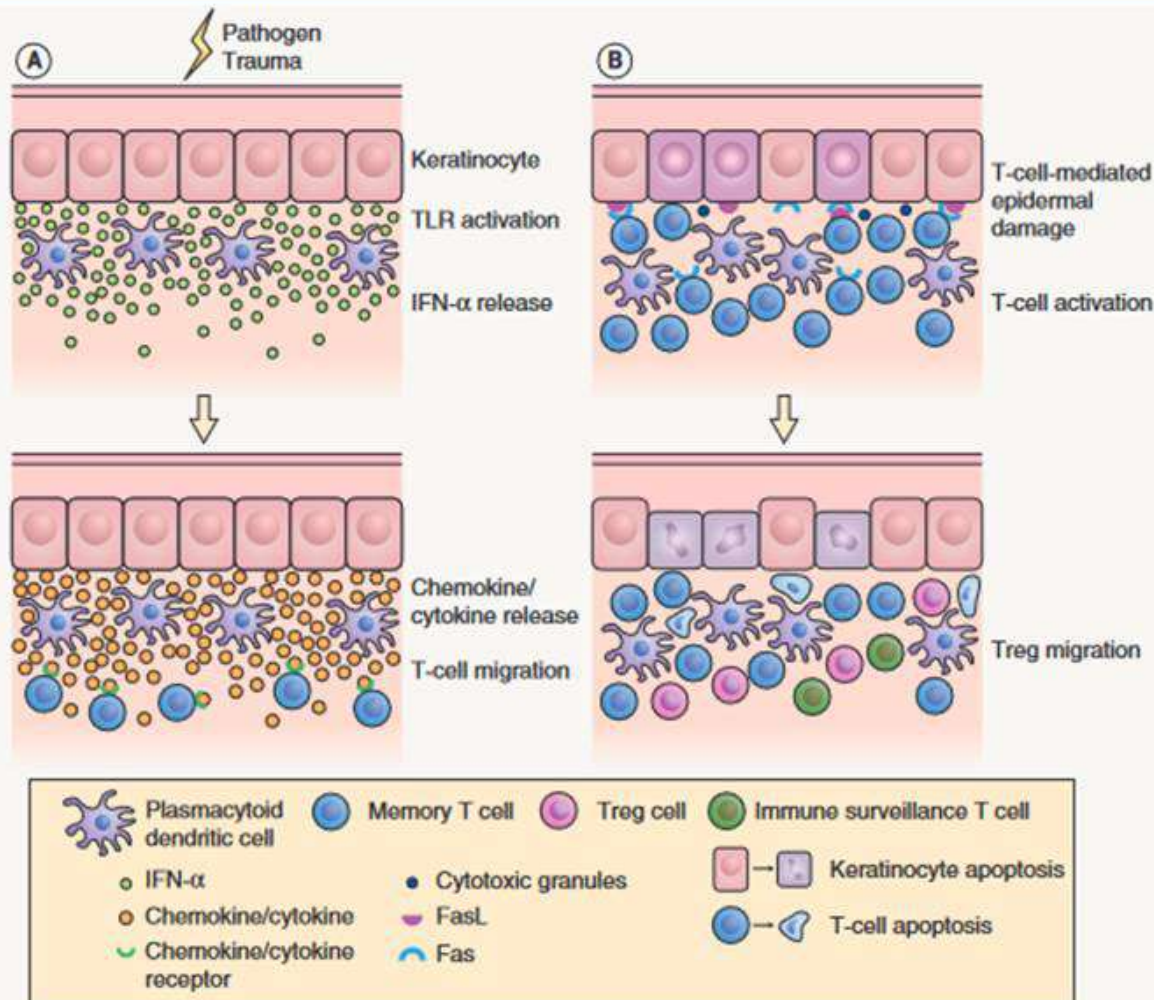
GENETIC PREDESPOSITION

- Familial LP (affects up to 10% of first-degree relatives of affected patients)
- Reports of concurrent LP in monozygotic twins
- Significant association between specific HLA antigens and LP.

IMMUNE DYSREGULATION:

- LP represents a T-cell-mediated autoimmune damage of basal keratinocytes that have been modified by triggers e.g. viral antigens or drugs).
 - Effector T cells: Cytotoxic CD8+ T cells.
 - Cytokines released and actions:
 - IFN- γ upregulates Fas (CD95, death receptor) expression by keratinocytes.
 - Fas ligand (Fas L) on CD8+ T-cells binds to Fas expressed on epidermal keratinocytes —▶ apoptosis.
 - - o Release of cytotoxic granules (perforin and granzyme B) —■>apoptosis.
- End result: Basal cell damage

PHASES OF LICHEN PLANUS



CLINICAL PICTURE (CLASSIC)

7 P
PAPULE
PRURITIC
Polygonal
Purple
Planer
Polished
Plentiful

Wickhams striae
Flexor , wrist , forearm , dorsum of
hand



HOW MANY VARIANTS OF LP?



SKIN
MMS :oral , genetal
HAIR
NAIL



ACTINIC
PIGMENTOSA
PALMOPLATER
INVERS



ANNULAR
LINEAR
ULCERATIVE
GUTTATE



- Hypertrophic – atrophic
- Bullous lp – lp pemphigoid
- lp _le overlapp
- Eruptive lp

2- ORAL LP



- May be only site
- 50% of cutaneous LP
- Types :
 - Reticular
 - Erosive : HCV
 - Leukoplasia
 - Others: atrophic , hypertrophic bullous , pigmented
- DD : oral ulceration e.g. pemphigus vulgaris, Behcet's disease, aphthous stomatitis.

2- GENITAL



- ✓ May alone or with oral , cutaneous .
- ✓ glans penis: annular , violaceous
 - ✓ DD: syphilis
- ✓ Vulvovaginal l.p: erosive
 - ✓ Scar , dyspareunia , malignancy
 - ✓ DD : LSEA : cicatricial pemphigoid

HAIR LP



- Keratotic plug surrounded by violaceous rim
- cicatricial alopecia
- variants :
 - FFA
 - Graham little syndrome
- DD: keratosis pilaris,
 - Darier's disease,
 - follicular mucinosis,
- lupus erythematosus

NAIL LP



- ☐ thinning
- ☐ Shedding
- ☐ Longitudinal line
- ☐ Pterygium
- ☐ 20 nail syndrome
- ☐ melanonychia

ACTINIC LP



- annular hyperpigmented patch surrounded by hypopigmented rim
- Sun exposed areas
- Variants
 - Classic
 - Annular
 - Melasma
 - Dyschromic
- DD: melasma

LP PIGMENTOSUS



- diffuse
- Intertiginous
- Face , neck
- Linear
- Skin type 3 , 4
- DD : ashy dermatosis

PALMOPLANTER LP



FIGURE 2: Symmetric, yellowish, hyperkeratotic plaques with a violaceous

➤ Yellowish , firm , non itchy papules , plaques surrounded by violaceous rim .

- DD
- SYPHILIS
- PS
- CALLOSITY
- WART



INVERSE LP



Fig. 11.11 Inverse lichen planus. Oval thin violaceous plaques in the axilla. Postinflammatory hyperpigmentation is also present. *Courtesy, Jeffrey P Callen, MD.*

- ✓ Intertriginous areas (axillae > inguinal)
- Pink to violaceous papules and plaques.
- DD: lp pigmentosus



Annular
Mainly penis , axilla
DD: GA



linear
Blaschko lines
Zosteriform
Not dermatomal



FIGURE 1: Brilliant erythematous plaques with extensive superficial

Ulcerative
Palmoplantar
Sole
scc



Successive crops
HCV
trunk



Fig. 11.10 Hypertrophic lichen planus. **A** On the shins, very thick discrete plaques with dyspigmentation are admixed with smaller linear plaques and areas of postinflammatory hyperpigmentation. **B** On the dorsal digits, thin violaceous plaques in addition to thick keratotic plaques that favor the knuckles.

B, Courtesy, Joyce Rico, MD.



Hypertrophic lp

Chin of tibia

Highly pruritic

Chronic

Sc

DD: LSC , amyloidosis, prurigo nodularis



Fig. 11.9 Unusual variants of lichen planus. **A** Atrophic lichen planus of the lower extremities. **B** Bullous lichen planus on the shin. **C** Lichen planus pemphigoides in a patient with anti-BP180 autoantibodies.

Resolving stag
DD: LSEA



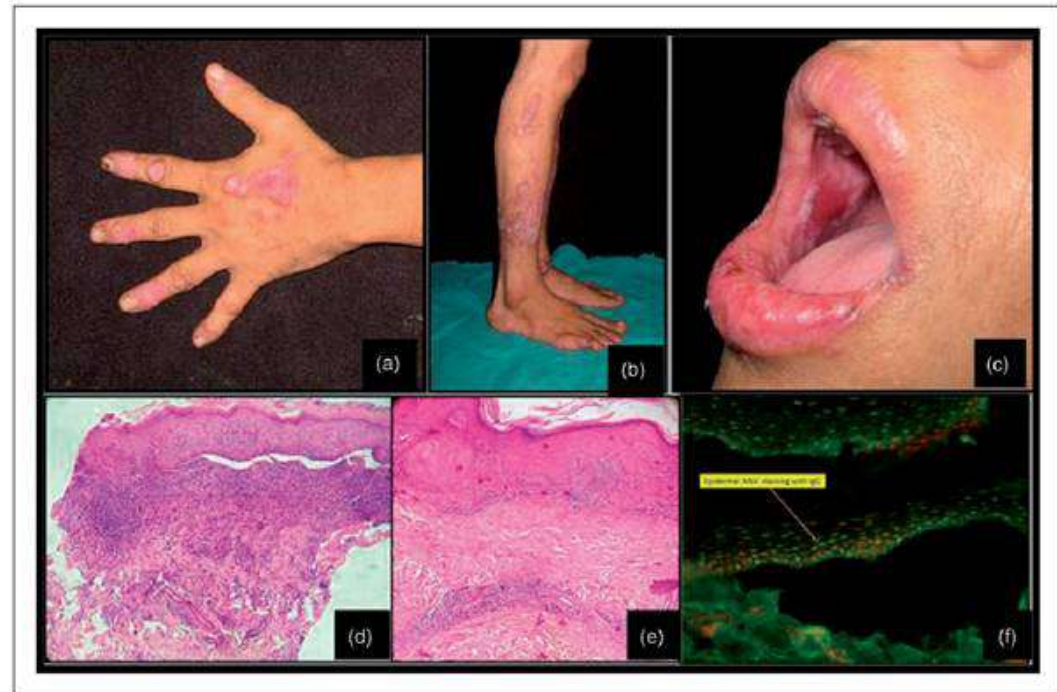
Bullous lp
Intense lichenoid infiltrate > severe
epidermal damage
> subepidermal bullae > bullae
within lp lesions
IF -ve



Lp pemphigoid
Damage of basal layer by lichen
> exposure to antigen > IgG
> BPA2, c 17
Bullae in uninvolved skin
IF +ve



LP , LE overlap
 Violaceous-to-erythematous patches
 and plaques with atrophy, mild scaling
 and telangiectasia usually in acral
 distribution.



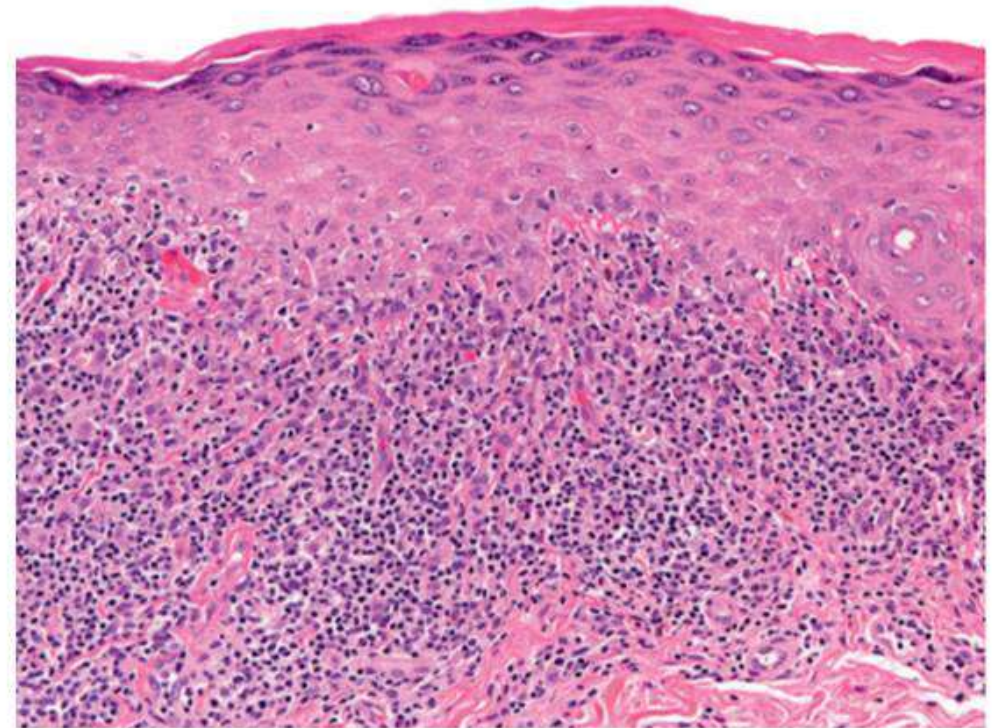
COMPLICATION

- skin :PIH,scaring ,atrophy
- Mucosa: scaring , stenosis
- Hair: cicatricial alopecia
- Nail :perminant damge
- SCC :oral , genital

Association
HCV , HBV
Liver disease
DM
Autoimmune disease
AA, VITILIGO

HISTOPATHOLOGY

- compact orthokeratosis
- Hypergranulosis :focal ,wedge shape
- Irrigular acanthosis:saw tooth
- Vacular degeneration of BMZ
- Separations(max joseph space
- Band like lymphocytic infiltrate
- Melanophages
- Civatte body



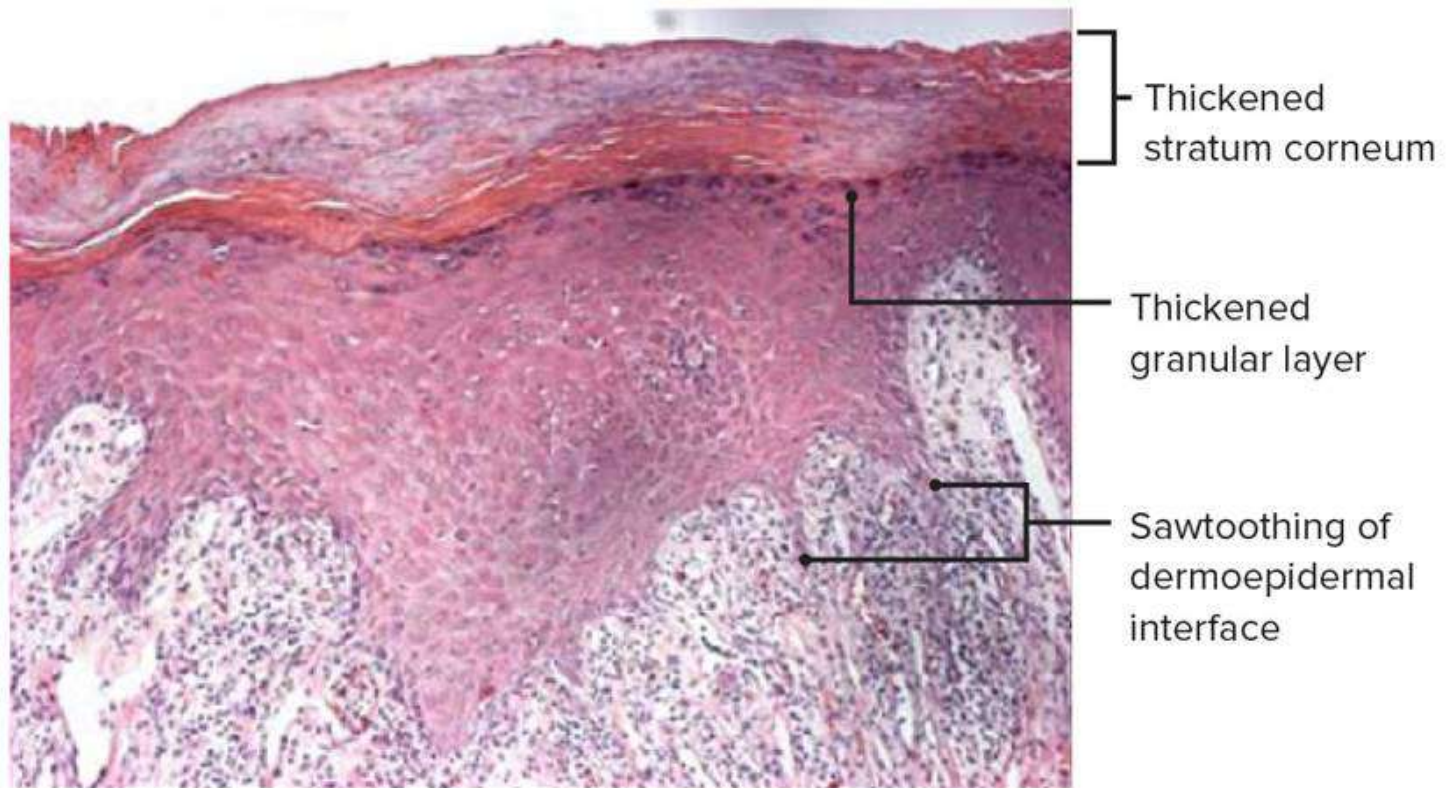
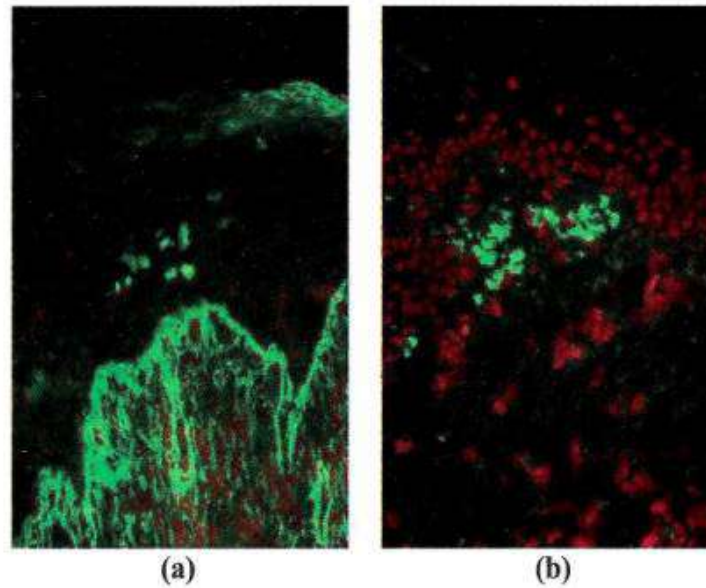


Fig 14.12 DIF of LP lesions (a) linear fibrin deposits at DEJ (b) globular IgM deposits (*McKee, 2012*).



DERMOSCOPY

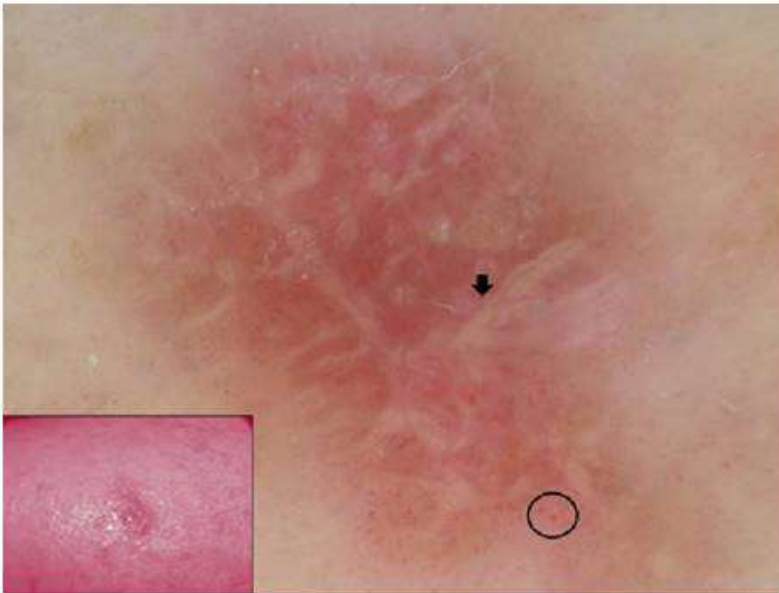
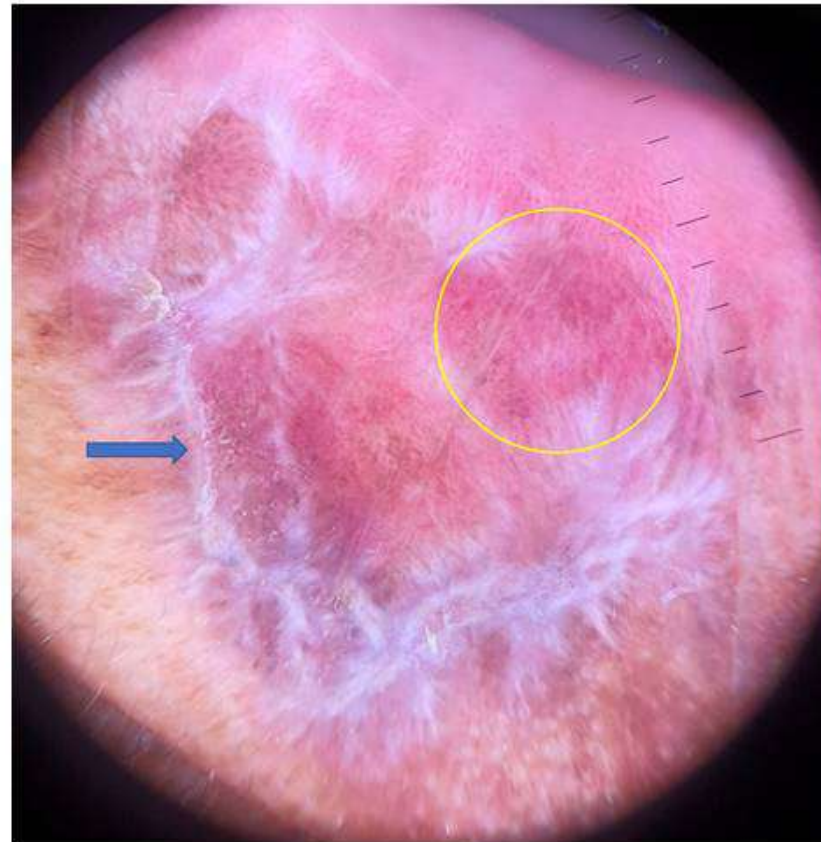


Figure 4.1. Clinical photograph with dermoscopy inset.



TREATMENT

THERAPEUTIC LADDER FOR LICHEN PLANUS



Topical corticosteroids, including under occlusion (2)
 Superpotent topical corticosteroids – oral LP (1); cutaneous LP (2)
 Topical calcineurin inhibitors – pimecrolimus and tacrolimus in oral LP (1);
 tacrolimus in vulvar LP (2) or other forms of LP (3)
 Moisturizers, including under occlusion (3)
 Intralesional corticosteroids (2)
 Intramuscular triamcinolone acetonide [0.5–1 mg/kg/month × 3–6 months] (3)
 Narrowband UVB (2)
 Oral metronidazole* [500mg po BID] (2)
 Antimalarials* (2)
 Systemic retinoids* – acitretin (1); alitretinoin (3)
 Griseofulvin* (2)
 PUVA (2)
 UVA1 (2)
 308-nm excimer laser – oral LP (2)
 Systemic corticosteroids† (1)
 Sulfasalazine* – cutaneous LP (1)
 Apremilast (2)
 Low-dose weekly methotrexate (2)
 Mycophenolate mofetil (2)
 Thalidomide (2)
 Azathioprine (3)
 Cyclosporine (3)
 Intravenous immunoglobulin (IVIg) (3)
 TNF- α inhibitors (3)
 Extracorporeal photochemotherapy (2)

*Implicated in lichenoid drug eruptions.

†Often a first-line therapy for severe, acute cutaneous LP.

*Especially with medications such as connect academy - BY DR. EMAN

THANK
you